TEAM-Focused CPR

AC-11 | Adult Cardiac

Quick flow

01 Recognize Arrest [EMT / AEMT / Paramedic]

Start CPR • apply pads/AED • BVM with oxygen • start scene timer

02 Assign Team Functions [EMT / AEMT / Paramedic]

Leader • compressor 1/2 • airway • monitor/defibrillator • IV/medications • recorder/timer

03 Deliver High-Quality CPR [EMT / AEMT / Paramedic]

100-120/min • depth 2-2.4 in • full recoil • compression fraction >80%

04 Manage Ventilation [EMT / AEMT / Paramedic]

30:2 before advanced airway • then continuous CPR with 1 breath every 6 sec • avoid hyperventilation

05 Two-Minute Cycle [EMT / AEMT / Paramedic]

Precharge when possible • rhythm/pulse check ≤10 sec • shock if indicated • immediately resume CPR

06 Communicate + Record [EMT / AEMT / Paramedic]

Closed-loop orders • announce rhythms, shocks, medications, airway, $EtCO_2$, and times

07 ROSC or Termination [EMT / AEMT / Paramedic]

ROSC: AC-10/AC-13 • persistent arrest: evaluate AC-12

Overview

- Operational high-performance CPR pathway for coordinated adult cardiac-arrest care by Claiborne EMS.
- Use AC-11 with AC-03 and the applicable rhythm-specific protocol; this page assigns team functions and CPR-quality expectations without replacing the clinical arrest algorithm.

Provider-Level Team Actions

Every provider may fill more than one team function when staffing is limited; lifesaving BLS care begins immediately.

EMT

- Begin compressions, BVM ventilation, AED/monitor pad placement, scene timing, and event documentation without waiting for ALS.
- Rotate compressors every two minutes or sooner for fatigue and provide real-time feedback on rate, depth, recoil, and pauses.
- Repeat assigned tasks and treatment orders back to the team leader and announce when each is complete.

AEMT

- Perform all EMT actions and assume vascular-access/medication or airway functions as assigned.
- Establish IV access first; use IO when IV access is unsuccessful, not feasible, or would delay required arrest medication.
- Administer treatments authorized by the active cardiac-arrest protocol and announce medication, dose, route, and time.

Paramedic

- Serve as team leader when assigned, preserve global oversight, and avoid task fixation.
- Direct rhythm interpretation, manual defibrillation, advanced-airway decisions, medication timing, reversible-cause treatment, ROSC transition, and termination evaluation.

- Use waveform capnography and available CPR feedback to identify correctable ventilation or compression problems.

Indications

- Any adult cardiac arrest managed by two or more Claiborne EMS responders.
- Training, simulation, and quality-improvement review of adult cardiac-arrest team performance.

Assessment | Team assignment

- Identify one team leader and assign compressor 1, compressor 2, airway, monitor/defibrillator, IV/medications, and recorder/timer functions as personnel arrive.
- Combine functions when staffing is limited, but do not combine tasks in a way that interrupts compressions, delays defibrillation, or prevents the leader from maintaining oversight.
- Use predetermined equipment positioning and role cards when available.

Assessment | CPR quality

- Compress at 100-120/min to a depth of at least 2 inches while avoiding depth greater than 2.4 inches.
- Allow complete chest recoil, minimize interruptions, and target a chest-compression fraction greater than 80%.
- Change compressors every two minutes or sooner for fatigue; complete the change during the planned rhythm check in 10 seconds or less.
- Use an audiovisual CPR feedback device when available and correct low or decreasing $EtCO_2$ by reassessing CPR quality, airway, and ventilation.

Assessment | Airway and rhythm-cycle coordination

- Use 30:2 compressions to ventilations before placement of an advanced airway.
- With an advanced airway in place, provide continuous compressions and one breath every six seconds with continuous waveform capnography.
- Prepare the next intervention before each two-minute rhythm check and precharge the defibrillator during compressions when feasible.
- Keep rhythm and pulse checks at 10 seconds or less and immediately resume compressions after any shock or no-shock decision.

Treatment steps

- Begin high-quality CPR, BVM ventilation with oxygen, pad placement, and the event timer immediately.
- Assign team functions as personnel arrive; the team leader states the working rhythm pathway and the next expected intervention.
- Use closed-loop communication: address the assigned provider, give one clear instruction, require repeat-back, and announce completion.
- The recorder announces the approaching two-minute rhythm check and tracks CPR cycles, rhythms, shocks, medications, airway events, $EtCO_2$, ROSC, transport, and termination times.
- At each two-minute cycle, rotate compressors, perform a rhythm/pulse check in 10 seconds or less, deliver a shock when indicated, and immediately resume CPR.
- Continuously reassess compression quality, ventilation rate, pad contact, airway position, vascular access, medication timing, and reversible causes.
- After ROSC, deliberately transition to AC-10 and AC-13. When field termination is being considered, open AC-12.
- After the event, complete a brief team debrief, preserve monitor/CPR data when available, and restore equipment readiness.

Warnings / precautions

- The team leader must avoid performing a task that prevents global oversight.
- Do not interrupt compressions for IV/IO access, airway placement, medication preparation, or nonessential movement.
- Avoid excessive ventilation before and after advanced-airway placement.
- Do not allow compressor rotation, rhythm analysis, pulse checks, charging, or shock delivery to create a pause longer than 10 seconds.
- Do not delay movement when transport is indicated by the active arrest, special-circumstance, or termination protocol.

Clinical pearls

- Predetermined positioning and role cards reduce duplicated work and delays.
- The next compressor should be in position before the two-minute cycle ends.
- Charging during compressions and resuming compressions immediately after shock delivery shortens the peri-shock pause.
- Structured post-event feedback and debriefing can improve CPR quality and team performance.

Special populations | Limited staffing

- Prioritize compressions, defibrillation, and effective ventilation before access, advanced airway, or documentation tasks.
- Request additional personnel early and reassign functions explicitly as responders arrive.

Special populations | Arrest during movement or transport

- Stop vehicle movement when required to safely deliver manual CPR or defibrillation.
- Reconfirm team functions and equipment security before patient movement and after every transfer between surfaces.

Related pathways / contacts

AC-03 Adult Cardiac Arrest - Open for the primary adult cardiac-arrest algorithm.

AC-09 VF / Pulseless VT - Open for shockable VF/pVT arrest treatment.

AC-10 Post-Resuscitation Care - Open immediately after sustained ROSC.

AC-12 Termination of Resuscitation - Open when persistent adult OHCA meets evaluation criteria.

AC-13 Post-Arrest Temperature Management - Open after ROSC when the adult remains unresponsive to verbal commands.

TB-10 Traumatic Arrest - Open for cardiac arrest caused by trauma.

Source and references

American Heart Association. 2025 Guidelines for CPR and ECC: Adult Basic Life Support. | American Heart Association. 2025 Guidelines for CPR and ECC: Adult Advanced Life Support. | American Heart Association. 2025 Guidelines for CPR and ECC: Systems of Care. | Claiborne EMS adult cardiac-arrest, rhythm, airway, post-resuscitation, and termination protocols.

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